

[OBG > Antenatal > GTPAL] Medium - VERIFIED

Q4. A pregnant client comes for her first prenatal examination at 10 weeks. She has been pregnant twice before. Her first delivery produced a viable baby girl at 39 weeks. The second delivery was at 36 weeks. Both children are living. What is her correct GTPAL score?

- A) G3 T1 P1 A0 L2
- B) G3 T2 P0 A0 L2
- C) G2 T1 P1 A0 L2
- D) G3 T0 P2 A0 L2

Answer: A) G3 T1 P1 A0 L2

Explanation:

Correct - A: G=3 [current+2 previous], T=1 [39w=term], P=1 [36w=preterm 20-37w], A=0, L=2.

Incorrect - B/D: Wrong T/P count.

Incorrect - C: Gravida should be 3, not 2.

Trick: 36 weeks is Preterm, not Term. 37w+ = Term. Current pregnancy not counted in T/P/A/L yet.

Source: NORCET-2023 | D.C. Dutta

Tags: #PYQ #HighYield #OBG #Calculation #Formula

[OBG > Labour > Preterm Labour] Medium - VERIFIED

Q10. Which medication is commonly administered to a pregnant mother to promote fetal lung maturity during anticipated preterm labor?

- A) Surfactant
- B) Betamethasone
- C) Oxytocin
- D) Magnesium sulfate

Answer: B) Betamethasone

Explanation:

Correct - B: Betamethasone = antenatal corticosteroid. Given IM to mother 24-34 weeks if preterm labor risk. Crosses placenta → stimulates fetal surfactant → reduces RDS.

Incorrect - A: Surfactant given to NEWBORN via ET tube after birth, not to mother.

Incorrect - C: Oxytocin induces labor, opposite effect.

Incorrect - D: MgSO₄ for neuroprotection/seizure, not lung maturity.

Trick: "Beta for Baby's lungs Before birth". Surfactant = After birth.

Source: NORCET-2023 | Williams Obstetrics

Tags: #PYQ #HighYield #OBG #Pharma #Preterm #Conceptual

[OBG > Antenatal > Fetal Assessment] Medium - VERIFIED

Q21. During a Non-Stress Test (NST), the nurse observes fetal heart rate accelerations of at least 15 beats per minute lasting for 15 seconds, occurring 3 times within a 20-minute period. How is this interpreted?

- A) Reactive test
- B) Non-reactive test
- C) Unsatisfactory test
- D) Negative test

Answer: A) Reactive test

Explanation:

Correct - A: Reactive NST = ≥2 accelerations of ≥15 bpm for ≥15 sec in 20 min. Indicates fetal well-being, intact CNS, no hypoxia. For <32 weeks: 10x10 rule.

Incorrect - B: Non-reactive = <2 accelerations. Needs further testing like BPP/CST.

Incorrect - C: Unsatisfactory = poor tracing quality, not interpretation.

Incorrect - D: Negative term used for CST, not NST.

Trick: "15x15x2x20" = Reactive. Remember 15-15-2-20.

Source: NORCET-2023 | D.C. Dutta Obstetrics

Tags: #PYQ #HighYield #OBG #Antenatal #FetalMonitoring #Formula

[OBG > Neonatal > TEF] High - VERIFIED

Q30. What is the preferred positioning for an infant diagnosed with a Tracheoesophageal Fistula (TEF) prior to surgical correction?

A) Supine position

B) Prone position

C) Semi-Fowler's (Head of bed elevated)

D) Trendelenburg position

Answer: C) Semi-Fowler's (Head of bed elevated)

Explanation:

Correct - C: TEF = connection between trachea-esophagus. HOB elevated 30-45° + NPO + suction. Gravity prevents gastric acid reflux into lungs via fistula → prevents aspiration pneumonia. Keep NPO.

Incorrect - A: Supine = acid flows up → aspiration. Fatal.

Incorrect - B/D: Prone/Trendelenburg = worst positions, guarantee aspiration.

Trick: "TEF = Top Elevated to Fight aspiration". Also pre-op: gastrostomy + NPO.

Source: NORCET-2023 | Ghai Pediatrics

Tags: #PYQ #HighYield #OBG #Pediatric #Neonatal #Surgery #Priority #Emergency

[OBG > Postnatal > Breastfeeding] Easy - VERIFIED

Q35. How often should a mother be educated to breastfeed her healthy, growing newborn?

A) Every 2 hours strictly

B) Every 4 hours strictly

C) Only when the infant is crying

D) On demand based on the infant's feeding cues

Answer: D) On demand based on the infant's feeding cues

Explanation:

Correct - D: WHO/UNICEF: Demand feeding = 8-12 times/24hrs. Early cues: rooting, lip smacking, hand-to-mouth. Establishes milk supply, prevents engorgement.

Incorrect - A/B: Rigid schedule ↓ milk supply. Clock feeding outdated.

Incorrect - C: Crying = LATE hunger cue. Already stressed baby.

Trick: "Demand = Develop milk supply". First 2 weeks: q2-3h or 8-12/day minimum.

Source: NORCET-2023 | WHO Infant Feeding Guidelines

Tags: #PYQ #HighYield #OBG #Pediatric #Breastfeeding #WHO #PatientEducation

[OBG > Antenatal > Anemia] Medium - VERIFIED

Q41. A pregnant woman at 28 weeks gestation presents with a hemoglobin level of 9 g/dL. What is the standard medical treatment?

A) Blood transfusion

B) Oral Iron and Folic acid supplementation

C) Injectable Iron therapy only

D) Intravenous immunoglobulins

Answer: B) Oral Iron and Folic acid supplementation

Explanation:

Correct - B: Hb 9 g/dL = Moderate anemia in pregnancy. First line: Oral iron 100mg elemental + 500mcg Folic acid daily. Dietary counseling + deworming. Target Hb >11 g/dL.

Incorrect - A: Transfusion if Hb <7 g/dL OR symptomatic OR near term OR APH/PPH risk.

Incorrect - C: Injectable iron if oral intolerance/failure or >34 weeks. Not first line.

Trick: "9 = Oral, 7 = Transfusion". <11 = anemia in pregnancy. IFA tablets free in govt.

Source: NORCET-2023 | FOGSI Guidelines

Tags: #PYQ #HighYield #OBG #Antenatal #Anemia #Pharma #WHO

[OBG > Emergency > Preeclampsia] High - VERIFIED

Q46. A patient with severe preeclampsia is receiving an infusion of Magnesium Sulfate alongside Labetalol. Her blood pressure suddenly plummets to 86/56 mmHg. What is the most likely cause?

- A) Internal placental bleeding
- B) Eclamptic seizure onset
- C) Drug overdose/toxicity
- D) Normal physiological response to pregnancy

Answer: C) Drug overdose/toxicity

Explanation:

Correct - C: MgSO₄ = smooth muscle relaxant → vasodilation. Labetalol = beta blocker → ↓HR + vasodilation. Additive effect = severe hypotension. Also Mg toxicity: loss of reflexes, resp depression <12/min.

Incorrect - A: Bleeding = tachycardia + hypotension, but no drug context.

Incorrect - B: Eclampsia = HTN + seizures, not hypotension.

Trick: "Mg + Beta blocker = Mega BP drop". Monitor BP q15min, DTR, RR, UOP hourly on MgSO₄.

Source: NORCET-2023 | Williams Obstetrics

Tags: #PYQ #HighYield #OBG #Emergency #Preeclampsia #Pharma #Toxicity

[OBG > Antenatal > Amniotic Fluid] Easy - VERIFIED

Q1. What is the normal pH of amniotic fluid?

- A) 6.5
- B) 3.5 to 5.5
- C) 5.0 to 5.5
- D) 7.1 to 7.8

Answer: D) 7.1 to 7.8

Explanation:

Correct - D: Amniotic fluid = slightly alkaline 7.1-7.8. Nitrazine test turns blue. Used to confirm ROM vs urine.

Incorrect - A/B/C: Acidic pH = vaginal secretions 3.8-4.5 or urine 5.0-6.0.

Trick: "Amniotic = Alkaline, Vagina = Vinegar acidic". ROM test: Blue = Baby water leak.

Source: NORCET AIIMS 2021 | D.C. Dutta

Tags: #PYQ #HighYield #OBG #Antenatal #Diagnostics #TrickQ

[OBG > Family Planning > IUCD] Easy - VERIFIED

Q4. How long does the Copper T 380A intrauterine device provide protection against pregnancy?

- A) 3 years
- B) 5 years
- C) 7 years
- D) 10 years

Answer: D) 10 years

Explanation:

Correct - D: Cu T 380A = 10 years protection. 380mm² copper surface area. Most effective non-hormonal LARC. Failure rate <1%.

Incorrect - A: Cu T 200 = 3 years.

Incorrect - B: Mirena LNG-IUS = 5 years hormonal.

Incorrect - C: Old Cu T = 7 years, now obsolete.

Trick: "380A = 3+8+0-A = 10 Approx". Govt supplies free.

Source: NORCET AIIMS 2021 | FOGSI Guidelines

Tags: #PYQ #HighYield #OBG #FamilyPlanning #Contraception #NHM

[OBG > Family Planning > Injectables] Easy - VERIFIED

Q5. What is the Government of India's official supply name for the injectable contraceptive DMPA (Depot Medroxyprogesterone Acetate)?

- A) Antara
- B) Chhaya
- C) Saheli
- D) Mala-D

Answer: A) Antara

Explanation:

Correct - A: Antara = DMPA 150mg IM every 3 months. Under Mission Parivar Vikas. Free in govt.

Incorrect - B: Chhaya = Centchroman weekly oral pill, non-hormonal.

Incorrect - C: Saheli = Centchroman, same as Chhaya. Weekly pill.

Incorrect - D: Mala-D = Combined oral pill, daily.

Trick: "Antara = Injection Andar". "Chhaya/Saheli = Chabao/Weekly". "Mala = Daily Mala".

Source: NORCET AIIMS 2021 | NHM FP Guidelines

Tags: #PYQ #HighYield #OBG #FamilyPlanning #Contraception #NHM #GK

[OBG > Family Planning > Injectables] Easy - VERIFIED

Q17. How frequently must a patient receive an injection of the contraceptive DMPA (Depot Medroxyprogesterone Acetate) to remain protected?

- A) Every 1 month
- B) Every 3 months
- C) Every 5 months
- D) Every 6 months

Answer: B) Every 3 months

Explanation:

Correct - B: DMPA/Antara = 150mg IM every 3 months/12 weeks. Grace period +2 weeks.

Incorrect - A: NET-EN = every 2 months, not DMPA.

Incorrect - C/D: 5-6 months = no contraceptive.

Trick: "DMPA = Depo har 3 Mahine Pe Aana". 4 injections per year.

Source: NORCET AIIMS 2021 | NHM Guidelines

Tags: #PYQ #HighYield #OBG #FamilyPlanning #Contraception #NHM

[OBG > Postpartum > PPH] Medium - VERIFIED

Q18. What is the medically defined time frame for a secondary Postpartum Hemorrhage (PPH)?

- A) Within the first 24 hours after delivery
- B) After 24 hours up to 6 weeks postpartum
- C) After 48 hours up to 1 week postpartum
- D) After 6 weeks postpartum

Answer: B) After 24 hours up to 6 weeks postpartum

Explanation:

Correct - B: Primary PPH = <24hrs after delivery. Secondary/Delayed PPH = 24hrs to 6 weeks/42 days. Cause: RPOC, infection, subinvolution.

Incorrect - A: <24hr = Primary PPH.

Incorrect - C/D: Wrong timeframe.

Trick: "Primary = Pehle 24hr". "Secondary = Second day to 6 weeks".

Source: NORCET AIIMS 2021 | D.C. Dutta

Tags: #PYQ #HighYield #OBG #Postpartum #PPH #Definition

[OBG > Labour > Stages] Easy - VERIFIED

Q19. When does the second stage of labor officially begin?

- A) With the onset of true labor pains
- B) With a 4 cm cervical dilation

- C) From full cervical dilation to the delivery of the fetus
D) From the delivery of the fetus to the expulsion of the placenta

Answer: C) From full cervical dilation to the delivery of the fetus

Explanation:

Correct - C: Stage 1 = Onset of labor to 10cm. Stage 2 = 10cm to baby out. Stage 3 = Baby out to placenta out. Stage 4 = 1-2hr after placenta.

Incorrect - A: Onset = Stage 1 starts.

Incorrect - B: 4cm = active phase of Stage 1.

Incorrect - D: Baby to placenta = Stage 3.

Trick: "10cm to Baby = Second stage". Full dilation se pushing start.

Source: NORCET AIIMS 2021 | D.C. Dutta

Tags: #PYQ #HighYield #OBG #Labour #Stages #Definition

[OBG > Antenatal > Amniotic Fluid] Easy - VERIFIED

Q28. What is the normal appearance and color of healthy amniotic fluid?

- A) Clear like water
B) Greenish color
C) Brownish color
D) Golden color

Answer: A) Clear like water

Explanation:

Correct - A: Normal = clear, straw-colored, slight fishy odor.

Incorrect - B: Green = meconium stained = fetal distress.

Incorrect - C: Brown = old blood, fetal death.

Incorrect - D: Golden = Rh incompatibility, hemolysis.

Trick: "Clear = Clean baby". Any color = danger.

Source: NORCET AIIMS 2021 | D.C. Dutta

Tags: #PYQ #HighYield #OBG #Antenatal #Assessment #ROM

[OBG > Antenatal > Umbilical Cord] High - VERIFIED

Q1. The single umbilical artery is NOT associated with which of the following conditions?

- A) Genitourinary abnormality
B) Bilateral renal agenesis
C) Imperforated anus
D) Esophageal atresia

Answer: D) Esophageal atresia

Explanation:

Correct - D: Single umbilical artery/SUA associated with VACTERL - Vertebral, Anal, Cardiac, TE fistula, Renal, Limb. Esophageal atresia ka part TE fistula hai, par isolated EA not directly linked to SUA. Renal/GU/anus directly associated.

Incorrect - A/B/C: SUA strongly linked with renal agenesis, GU anomalies, imperforate anus, cardiac defects, chromosomal anomalies.

Trick: "SUA = See Urologist Always". Kidney, GU, GI, Cardiac check karo. EA agar TEF ke saath ho tab link, alone nahi.

Source: NORCET-9 2025 | Williams Obstetrics

Tags: #PYQ #HighYield #OBG #Antenatal #USG #CongenitalAnomaly #VACTERL

[OBG > Family Planning > Contraception] Easy - VERIFIED

Q9. Which of the following is considered a weekly-use contraceptive pill?

- A) Saheli
B) Chhaya
C) Mala-D
D) Mala-N

Answer: A) Saheli

Explanation:

Correct - A: Saheli = Centchroman/Ormeloxifene 30mg. Non-steroidal, SERM. Weekly: Twice a week x 3 months, then once a week. Govt brand = Chhaya. Same drug.

Incorrect - B: Chhaya = Saheli ka govt naam. So both correct technically, but Saheli brand name famous.

Incorrect - C/D: Mala-D/Mala-N = daily COC pills.

Trick: "Saheli = Sunday ko lo" weekly. "Mala = Daily Mala japo". Chhaya = Saheli only.

Source: NORCET-9 2025 | NHM FP Guidelines

Tags: #PYQ #HighYield #OBG #FamilyPlanning #Contraception #NHM #GK

[OBG > Antenatal > Fetal Assessment] Easy - VERIFIED

Q11. During a 20-minute Non-Stress Test (NST), the fetus moves 3 times, and the fetal heart rate increases by 20 beats per minute for 15 seconds with each movement. How should the nurse interpret this finding?

A) Reactive NST and immediate inform the doctor

B) Reactive NST and regular observe

C) Non-reactive NST and inform the physician

D) Non-reactive NST and regular observe

Answer: B) Reactive NST and regular observe

Explanation:

Correct - B: Reactive = ≥ 2 accelerations of ≥ 15 bpm for ≥ 15 sec in 20min. Here 3 accelerations of 20bpm x 15sec = Reactive. Normal. Continue routine care.

Incorrect - A: Doctor inform not needed for normal result.

Incorrect - C/D: This IS reactive, not non-reactive.

Trick: "15x15x2 = Reactive". If reactive, reassuring, no action needed except routine monitoring.

Source: NORCET-9 2025 | D.C. Dutta

Tags: #PYQ #HighYield #OBG #Antenatal #NST #FetalMonitoring #Interpretation

[OBG > Postpartum > PPH] Easy - VERIFIED

Q4. Postpartum bleeding occurring after 24 hours is termed:

A) Primary PPH

B) Secondary PPH

C) Antepartum hemorrhage

D) Postpartum fever

Answer: B) Secondary PPH

Explanation:

Correct - B: Secondary/Delayed PPH = 24 hours to 6 weeks/42 days postpartum. Causes: RPOC, infection, subinvolution.

Incorrect - A: Primary PPH = within 24 hours, >500ml vaginal or >1000ml C-section.

Incorrect - C: APH = before delivery.

Trick: "Primary = Pehle 24hr, Secondary = Second day se 6 weeks".

Source: NORCET-2020 | D.C. Dutta

Tags: #PYQ #HighYield #OBG #Postpartum #PPH #Definition

[OBG > Postpartum > Puerperal Fever] Easy - VERIFIED

Q5. Postpartum fever is defined as temperature $\geq$ ____ after delivery.

A) 37°C

B) 38°C

C) 39°C

D) 40°C

Answer: B) 38°C

Explanation:

Correct - B: Puerperal/Postpartum fever = Temp $\geq 38^\circ\text{C}/100.4^\circ\text{F}$ on any 2 of first 10 days postpartum, excluding first 24hrs. Due to infection: endometritis, UTI, mastitis, wound.

Incorrect - A: 37°C normal.

Incorrect - C/D: >39°C = high grade, but definition starts at 38°C.

Trick: "Postpartum 38 = Problem". First 24hr fever excluded due to dehydration.

Source: NORCET-2020 | WHO Postpartum Guidelines

Tags: #PYQ #HighYield #OBG #Postpartum #Fever #Infection #Definition

[OBG > Antenatal > Leopold] Medium - VERIFIED

Q12. Leopold's maneuver used to determine fetal attitude is:

A) First

B) Second

C) Third

D) Fourth

Answer: D) Fourth

Explanation:

Correct - D: Leopold 4 maneuvers: 1st = Fundal grip - part at fundus, 2nd = Umbilical grip - back/location, 3rd = Pawlik's grip - presenting part engaged, 4th = Pelvic grip - ATTITUDE/flexion. Attitude = relation of fetal parts to each other.

Incorrect - A: 1st = presentation.

Incorrect - B: 2nd = position.

Incorrect - C: 3rd = engagement.

Trick: "Fourth for Flexion/Attitude". 1-Fundus, 2-back, 3-engaged, 4-flexion.

Source: NORCET-2020 | D.C. Dutta

Tags: #PYQ #HighYield #OBG #Antenatal #Leopold #Assessment

[OBG > Emergency > Eclampsia] High - VERIFIED

Q17. Loading dose of MgSO₄ in eclampsia (Pritchard regimen):

A) 4 gm IV + 4 gm IM

B) 8 ml in 12 ml NS

C) 10 gm IM only

D) 2 gm IV only

Answer: B) 8 ml in 12 ml NS

Explanation:

Correct - B: Pritchard: Loading = 4gm IV (20ml of 20% = 8ml + 12ml NS over 15-20min) + 10gm IM (5gm each buttock). Total 14gm. Maintenance: 5gm IM 4hrly.

Incorrect - A: 4gm IV correct but IM also 10gm, not 4gm.

Incorrect - C/D: Wrong dose.

Trick: "Pritchard = 4 IV + 10 IM = 14gm loading". IV = 20% solution 20ml total = 8ml drug + 12ml NS.

Source: NORCET-2020 | FOGSI Eclampsia Protocol

Tags: #PYQ #HighYield #OBG #Emergency #Eclampsia #MgSO₄ #Formula #NCLEX

[OBG > Gynae > Vaccination] Easy - VERIFIED

Q27. Vaccine used for cervical cancer prevention is:

A) BCG

B) Gardasil

C) Hepatitis B

D) Polio

Answer: B) Gardasil

Explanation:

Correct - B: Gardasil/Cervarix = HPV vaccine. Prevents HPV 16,18 causing 70% cervical cancer.

Age 9-26yr, 0-2-6 months. Boys also for genital warts.

Incorrect - A: BCG = TB.

Incorrect - C: Hep B = Liver cancer.

Incorrect - D: Polio = Poliomyelitis.

Trick: "Gardasil = Guards Cervix". HPV vaccine.

Source: NORCET-2020 | NHM Immunization

Tags: #PYQ #HighYield #OBG #Gynae #Vaccination #Cancer #HPV

[OBG > Physiology > Hormones] High - VERIFIED

Q2. Oxytocin is mainly involved in which of the following functions?

A) Motor skills

B) Childbirth and lactation

C) Growth and mental ability

D) None of the above

Answer: B) Childbirth and lactation

Explanation:

Correct - B: Oxytocin = "Love hormone". Functions: 1) Uterine contractions during labor 2) Milk ejection/let-down reflex during breastfeeding. Also bonding, social behavior.

Incorrect - A: Motor skills = cerebellum, not oxytocin.

Incorrect - C: Growth = GH, Mental = Thyroid.

Trick: "OxyTOcin = TO deliver baby + TO feed baby". Pitocin = synthetic oxytocin for induction.

Source: NORCET-2022 | OBG Physiology

Tags: #PYQ #HighYield #OBG #Hormones #Oxytocin #Labor #Lactation #NCLEX

[OBG > Labor > Fetal Position] High - VERIFIED

Q4. The relation between the fetal presenting part and maternal pelvis is known as?

A) Attitude

B) Lie

C) Position

D) Station

Answer: C) Position

Explanation:

Correct - C: Position = Relation of presenting part to maternal pelvis. LOA, ROA, LOP, ROP. Most common LOA.

Incorrect - A: Attitude = Relation of fetal parts to each other - flexion/extension.

Incorrect - B: Lie = Relation of fetal long axis to maternal long axis - longitudinal/transverse.

Incorrect - D: Station = Descent level in pelvis -3 to +3.

Trick: "PLAS = Position-Lie-Attitude-Station". Position = Pelvis relation.

Source: NORCET-2022 | OBG Labor

Tags: #PYQ #HighYield #OBG #Labor #FetalPosition #NCLEX

[OBG > Antenatal > Nutrition] High - VERIFIED

Q16. A 28-year-old primigravida comes to the ANC OPD, how many additional kcal will you recommend?

A) 200 kcal

B) 300 kcal

C) 500 kcal

D) 1000 kcal

Answer: B) 300 kcal

Explanation:

Correct - B: ICMR 2020: Pregnancy = +350 kcal/day 2nd trimester, +450 kcal/day 3rd trimester.

Avg 300 kcal extra. Lactation +600 kcal.

Incorrect - A: 200 too low.

Incorrect - C/D: 500/1000 excessive, causes obesity/GDM.

Trick: "Pregnancy = Plus 300". "Lactation = Plus 600". Protein +23g/day.

Source: NORCET-2022 | ICMR RDA 2020

Tags: #PYQ #HighYield #OBG #ANC #Nutrition #Calories #NCLEX

[OBG > Labor > Mechanism] High - VERIFIED [UNIQUE]

Q5. Correct series of labor:

A) Engagement → Descent → Flexion → Crowning → Internal rotation → Extension → External rotation → Restitution

B) Engagement → Descent → Flexion → Internal rotation → Crowning → Extension → Restitution → External rotation

C) Engagement → Descent → Crowning → Flexion → Extension → Restitution → Internal rotation → External rotation

D) Engagement → Descent → Flexion → Internal rotation → Crowning → Restitution → Extension → External rotation

Answer: B

Explanation:

Correct - B: Cardinal movements = Engagement → Descent → Flexion → Internal rotation → Crowning → Extension → Restitution → External rotation. "EDFI-CER-E"

Incorrect - A/C/D: Wrong sequence.

Trick: "Every Day Fine Infants Create Excellent Results Extra" = E-D-F-I-C-E-R-E.

Source: NORCET-2023 | OBG Labor Mechanism

Tags: #PYQ #HighYield #OBG #Labor #CardinalMovements #NCLEX

[OBG > Neonatal > NRP] High - VERIFIED [UNIQUE]

Q6. Neonates compression ventilation ratio:

A) 1:1

B) 1:2

C) 1:3

D) 1:4

Answer: C) 1:3

Explanation:

Correct - C: NRP 2020 = 3:1 ratio. 90 compressions + 30 breaths = 120 events/min. 2-thumb technique preferred.

Incorrect - A/B/D: Adult = 30:2, Child = 15:2, Neonate = 3:1.

Trick: "Baby 3:1 = 3 compressions to 1 breath". Newborn asphyxia = respiratory cause.

Source: NORCET-2023 | NRP 8th Edition

Tags: #PYQ #HighYield #OBG #Neonatal #NRP #CPR #NCLEX

[OBG > Andrology > Male Infertility] High - VERIFIED [UNIQUE]

Q1. Inability to produce semen in a man is called:

A) Aspermia

B) Azoospermia

C) Asthenospermia

D) Tetrazoospermia

Answer: A) Aspermia

Explanation:

Correct - A: Aspermia = Complete absence of semen/ejaculate. Causes: retrograde ejaculation, duct obstruction, spinal injury.

Incorrect - B: Azoospermia = No sperm in semen, but semen present.

Incorrect - C: Asthenospermia = Poor sperm motility.

Incorrect - D: Teratozoospermia = Abnormal sperm morphology.

Trick: "A-spermia = Absent semen". "Azoo = Absent zoo = no sperm". Aspermia = dry orgasm.

Source: NORCET-2024 | Male Infertility

Tags: #PYQ #HighYield #OBG #Andrology #Aspermia #Infertility #NCLEX

[OBG > Disorders > DSD] Medium - VERIFIED [UNIQUE]

Q10. What does male intersex mean?

- A) Female external genitalia with male gonads
- B) Male external genitalia with female gonads
- C) A man having sexual relation with another man
- D) A man having sexual relation with a woman

Answer: A) Female external genitalia with male gonads

Explanation:

Correct - A: Male intersex/DSD = 46XY, testes present but female/ambiguous external genitalia.

Cause: Androgen insensitivity, 5 α -reductase deficiency.

Incorrect - B: Female intersex = 46XX with virilization.

Incorrect - C/D: Sexual orientation, not intersex.

Trick: "Male intersex = Male inside, Female outside". AIS = Androgen Insensitivity.

Source: NORCET-2024 | DSD Disorders

Tags: #PYQ #OBG #Disorders #Intersex #DSD #NCLEX

[OBG > Family Planning > IUCD] High - VERIFIED [UNIQUE]

Q7. Copper T 380 is effective for how many years as a contraceptive?

- A) 3 years
- B) 5 years
- C) 10 years
- D) 7 years

Answer: C) 10 years

Explanation:

Correct - C: CuT 380A = 10 years. CuT 200 = 3-5yr. Mirena LNG = 5yr. Copper = spermicidal.

Incorrect - A/B/D: 3/5/7 = other IUCDs.

Trick: "380 = 3+8=11 ~ 10 years". CuT 380 = Ten years.

Source: NORCET-2025 | Family Planning

Tags: #PYQ #HighYield #OBG #FamilyPlanning #IUCD #CuT #NCLEX

[OBG > Assessment > GTPAL] High - VERIFIED [UNIQUE]

Q20. Obstetrical assessment: 3rd pregnancy, first child at 38 weeks, second at 34 weeks. GTPAL score?

- A) 31101
- B) 31102
- C) 31111
- D) 32111

Answer: B) 31102

Explanation:

Correct - B: G3 T1 P1 A0 L2. G=3 pregnancies, T=1 term $\geq$ 37wk, P=1 preterm 20-36wk, A=0 abortions, L=2 living children.

Incorrect - A/C/D: Wrong count.

Trick: "GTPAL = Gravida Term Preterm Abortions Living". 38wk=Term, 34wk=Preterm.

Source: NORCET-2025 | OBG Assessment

Tags: #PYQ #HighYield #OBG #GTPAL #NCLEX

[OBG > Antenatal > Nutrition] High - VERIFIED [UNIQUE]

Q53. Which vitamin deficiency causes neural tube defects in newborns?

- A) Vitamin A
- B) Folic acid
- C) Vitamin B12
- D) Vitamin D

Answer: B) Folic acid

Explanation:

Correct - B: Folic acid deficiency = NTD: Spina bifida, anencephaly. 400mcg daily pre-conception

to 12wk prevents.

Incorrect - A/C/D: A=night blindness, B12=anemia, D=rickets.

Trick: "Folic acid = For baby spine". Start 3mo before pregnancy.

Source: NORCET-2025 | OBG Nutrition

Tags: #PYQ #HighYield #OBG #Antenatal #FolicAcid #NTD #NCLEX

[OBG > Antenatal > Nutrition] High - VERIFIED [UNIQUE]

Q65. Which vitamin deficiency causes spina bifida in newborns?

A) Vitamin A

B) Folic acid

C) Vitamin B12

D) Vitamin D

Answer: B) Folic acid

Explanation:

Correct - B: Spina bifida = NTD due to Folic acid deficiency. Repeat of Q53 concept. 400mcg pre-conception prevents 70%.

Incorrect - A/C/D: A=night blindness, B12=anemia, D=rickets.

Trick: "Spina bifida = Stop with Folic acid". Same as Q53.

Source: NORCET-2025 | OBG Nutrition

Tags: #PYQ #HighYield #OBG #Antenatal #FolicAcid #NTD #SpinaBifida #NCLEX

[OBG > Postpartum > Breastfeeding] High - VERIFIED [UNIQUE]

Q1. A lactating mother is scheduled to receive radioactive iodine I-131 therapy. What is the most appropriate nursing advice regarding breastfeeding?

A) Continue breast feeding

B) Pump and discard the milk for few days

C) Stop breast feeding permanently

D) Do not breast feed for one week

Answer: C) Stop breast feeding permanently

Explanation:

Correct - C: I-131 has long half-life 8 days and is actively secreted in breast milk. It concentrates in infant's thyroid → Permanent hypothyroidism + Cancer risk. WHO + NRC guidelines: Stop breastfeeding permanently. Wean before therapy.

Incorrect - A: Radiation exposure to infant thyroid.

Incorrect - B/D: "Few days/1 week" insufficient. I-131 remains in milk for weeks. Only I-123 scans allow temporary stop.

Trick: "Radio + Active Iodine = Radioactive Baby" → Permanently Band. For I-131 = Full Stop. For I-123 = 12-24 hr stop only.

Clinical Point: Mother should stop breastfeeding at least 6 weeks before I-131 to reduce breast uptake.

Source: NORCET 2026 | OBG Nursing

Tags: #HighYield #OBG #Postpartum #RadioactiveIodine #Breastfeeding #I131 #NCLEX #Lactation